



JDScience BTEC Health and Social Care Unit 2 Set B mark scheme

Original JDScience mark scheme. Indicative content only. Equivalent wording that meets the command word should be credited. Not an official Pearson paper.

Use this booklet after the matching practice paper. Do not issue it with the student paper.

Time: 1 hour 30 minutes · **Total marks:** 80

Question 1 · 20 marks

Riverside Children's Centre offers stay-and-play, health visitor clinics, and parenting groups. Next door is an NHS midwifery team. A local food bank (voluntary) operates on Fridays. Some families also pay for a private baby-massage class in the hall.

(a) [2 marks]

Identify two services that are part of the NHS in this case.

- Health visitor clinics; midwifery team.

(b) [4 marks]

Describe how a children's centre can promote the health of families.

- Early contact, child development advice, immunisation clinics, reducing isolation, signposting to food bank or social care.

(c) [6 marks]

Explain the advantages and disadvantages of using a private baby-massage class alongside NHS care.

- Advantages: extra choice, possibly smaller groups, convenient if NHS groups are full.
- Disadvantages: cost excludes some families; not a replacement for clinical midwifery/health visiting; quality varies.

(d) [8 marks]

Discuss why partnership between statutory and voluntary organisations matters in this community.

- Food bank meets immediate need NHS does not provide; centres can refer with consent.
- Shared aims: children's wellbeing; different funding and rules.
- Risks of duplication or families falling through gaps without communication.
- Level 3: applied discussion with a conclusion.

Question 2 · 20 marks

Lina is a newly qualified adult nurse on a medical ward. She works with a consultant, a healthcare assistant, a discharge coordinator and a hospital social worker. A patient, Mr Cole, is medically fit but cannot manage stairs at home.

(a) [2 marks]

Identify two roles that are mainly social care rather than clinical nursing.

- Hospital social worker; possibly discharge coordinator if arranging social care (accept if justified). Healthcare assistant is health support.

(b) [4 marks]

Outline the nurse's responsibilities when preparing Mr Cole for discharge.

- Share clinical information with the team (consent); check understanding of medication; raise concerns about stairs; follow discharge policy.

(c) [6 marks]

Explain how effective teamwork could prevent a failed discharge.

- Social worker/OT assess home and stairs; equipment or interim care; consultant agrees medical fitness; nurse notices day-to-day ability.
- Without teamwork he may return as an emergency.

(d) [8 marks]

Evaluate the impact of poor communication between professionals on people who use services.

- Missed information → unsafe discharge, repeated admissions, distress.
- Families get mixed messages; trust falls.
- Good handover, records and MDT meetings reduce harm.
- Level 3: evaluation with examples applied to Mr Cole and a judgement.

Question 3 · 20 marks

Grace is 45 and has been diagnosed with bipolar disorder. She uses a community mental health team (CMHT), her GP, and a peer-support charity. She wants to return to part-time work. She is worried colleagues will treat her unfairly.

(a) [2 marks]

Identify two organisations that could support Grace's mental health.

- CMHT; GP; peer-support charity. Award two.

(b) [4 marks]

Describe two ways anti-discriminatory practice should be shown if Grace returns to work.

- Equality Act: reasonable adjustments, no harassment; confidential handling of health information; focus on ability not stereotype.

(c) [6 marks]

Explain how empowerment can be promoted in Grace's care.

- Shared decision-making about medication and work; peer support as lived experience; advocacy if needed; not making choices for her.

(d) [8 marks]

Discuss barriers people with mental health needs may face when accessing work and care.

- Stigma, fear of disclosure, fluctuating needs, appointment times, waiting lists, financial stress.
- Supports: Access to Work, occupational health, flexible CMHT appointments.
- Level 3: barriers and how they might be reduced, applied to Grace.

Question 4 · 20 marks

A residential care home has a medication policy, a complaints procedure and an equality and diversity policy. A resident, Mrs Adeyemi, says staff ignore her when she asks to wear her own clothes to the dining room. A student on placement hears this.

(a) [2 marks]

Identify two rights Mrs Adeyemi has as a person using the service.

- Dignity; choice; respect for culture/identity; to complain; to be safe; to be involved in decisions.

(b) [4 marks]

Describe how the complaints procedure should be used.

- Listen, record, try informal resolution, escalate per policy, no victimisation, timescales, independent advocacy if needed.

(c) [6 marks]

Explain how the student should respond, with reference to professional behaviour.

- Do not ignore; support Mrs Adeyemi to speak to a senior; follow placement reporting; do not gossip; if harm/neglect suspected, safeguarding.

(d) [8 marks]

Evaluate how well policies alone can protect people who use services.

- Policies set standards and give routes (complaints, equality).
- They fail if staff are rushed, untrained or if culture dismisses residents' wishes (own clothes).
- Inspection, training, leadership and person-centred culture make policies real.
- Level 3: judgement that policies are necessary but not sufficient.